**Clinical Study Summary Report (CSSR)**
**Document Status:** Final | **Version:** 1.0 | **Date:** 02-Apr-2026

---

## 1. Administrative & Study Identification

**Full Study Title:** A Phase IB, Open-Label, Multi-Center Study to Determine the Efficacy and Safety of Durvalumab and/or Novel Oncology Therapies, With or Without Chemotherapy, for First-Line Stage IV Non-Small Cell Lung Cancer (NSCLC)
**Brief Study Title:** A Study of Novel Anti-cancer Agents in Patients With Previously Untreated NSCLC
**Short Title/Acronym:** MAGELLAN
**Protocol Number:** D933IC00001
**NCT Number:** NCT03819465
**Posting ID:** 987019807463995285
**Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING)
**Sponsor/Company Name:** AstraZeneca
**Investigational Product:** Durvalumab, Danvatirsen, Oleclumab, MEDI5752, AZD2936, and Chemotherapy (Pemetrexed, Carboplatin, Gemcitabine, Cisplatin [Platinol; ATC: L01XA01], Nab-paclitaxel)
**Phase of Development:** Phase 1 (Phase 1B)
**Medical Monitor:** AstraZeneca Clinical Operations Lead

---

## 2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** To determine the safety and tolerability of durvalumab and/or novel oncology therapies, with or without chemotherapy, for first-line Stage IV Non-Small Cell Lung Cancer (NSCLC). **Secondary Objectives:** To evaluate the preliminary anti-tumor activity (Objective Response Rate [ORR], Duration of Response [DoR], Progression-Free Survival [PFS]), as well as pharmacokinetics (PK) and immunogenicity of the novel agents and combinations.

**2.2 Background & Rationale** Standard-of-care therapies for metastatic non-small cell lung cancer (mNSCLC) have mixed outcomes that remain unsatisfactory for many patients. Combining immune-checkpoint inhibitors like durvalumab (anti-PD-L1) with novel agents (e.g., oleclumab [anti-CD73], danvatirsen) or chemotherapy might provide a synergistic anti-tumor effect by antagonizing separate immune pathways or modulating the tumor microenvironment. This study aims to evaluate these combinations in treatment-naïve advanced NSCLC patients.

---

## 3. Study Design & Methodology

**3.1 Design Framework Study Type:** Interventional **Control Type:** Active Comparator / Parallel experimental arms **Blinding:** Open-label **Randomization:** Parallel Assignment **Trial Status:** Active, not recruiting

**3.2 Planned Enrollment & Duration Target $N$:** 175 (Targeted/ Actual enrollment) **Number of Sites:** 41 locations globally (e.g., USA, Canada, South Korea, Taiwan, Poland, Spain, Belgium, Austria, Thailand) **Estimated Study Start:** 27-Dec-2018 **Estimated Primary Completion:** 23-May-2023

---

## 4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Histologically or cytologically documented Stage IV NSCLC not amenable to curative surgery or radiation. 2. No prior chemotherapy or any other systemic therapy for metastatic NSCLC. 3. Prior platinum-containing adjuvant, neoadjuvant, or definitive chemoradiation for advanced disease are eligible, if progression has occurred >12 months from end of last therapy. 4. Known tumor PD-L1 status. 5. Tumors that lack activating EGFR mutations and ALK fusions or documented local test result for any other known genomic alteration for which a targeted therapy is approved in first line per local standard of care. 6. WHO/ECOG performance status of 0 or 1 at enrollment. 7. Life expectancy of at least 12 weeks. 8. Troponin I or T ≤ ULN (per institutional guidelines).

**4.2 Exclusion Criteria** 1. Active or prior documented autoimmune or inflammatory disorders. 2. History of active primary immunodeficiency. 3. Any prior chemotherapy or any other systemic therapy for metastatic NSCLC. 4. Untreated central nervous system (CNS) metastases.

---

## 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** Multi-arm study. Example combination (Arms B1/B3): Durvalumab 1500 mg IV every 3 weeks (Q3W) for four cycles, then every 4 weeks (Q4W) starting at Cycle 5 Day 1, alone or in combination with chemotherapy (including Cisplatin/Platinol) ± novel agents (e.g., Oleclumab, Danvatirsen, MEDI5752, AZD2936). **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Treatment continued until objective disease progression, unacceptable toxicity, withdrawal of consent, or other discontinuation criteria are met.

**5.2 Concomitant & Prohibited Therapy Permitted:** Standard supportive care, localized palliative radiotherapy for symptom control. **Prohibited:** Other investigational anti-cancer therapies, concurrent systemic immunosuppressive medications.

---

## 6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures |
| :--- | :--- | :--- |
| **Screening** | Day -28 to 0 | Informed Consent, Medical History, Tumor tissue testing (PD-L1/genomics), Baseline Imaging |
| **Treatment** | Cycles 1-4 | Dosing of Durvalumab/Chemo/Novel agent combinations (Q3W), Safety Labs, PK/PD sampling |
| **Maintenance** | Cycle 5+ | Dosing of Durvalumab/novel combinations (Q4W), Tumor Assessments |
| **End of Study** | +30 Days | Final Vital Signs, Exit Interview, Safety Follow-Up, Survival Follow-Up |

---

## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoint Definition:** Safety and tolerability as assessed by the incidence of Treatment-Emergent Adverse Events (TEAEs) assessed by CTCAE v5.0, Serious Adverse Events (SAEs), and Dose-Limiting Toxicities (DLTs). **Measurement Method:** Clinical assessment evaluated by the investigator according to CTCAE criteria.

**7.2 Secondary & Exploratory Endpoints** * Objective Response Rate (ORR) per RECIST v1.1. * Duration of Response (DoR) and Progression-Free Survival (PFS). * Single and multiple-dose Pharmacokinetics (PK). * Immunogenicity (e.g., incidence of anti-drug antibodies [ADAs]).

---

## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Standard collection and assessment of clinical and laboratory adverse events continuously from screening through the follow-up period. **Serious Adverse Events (SAEs):** Investigational site must report all SAEs to the sponsor within 24 hours of awareness. **Data Monitoring Committee (DMC):** A Safety Review Committee (SRC) evaluates dose-limiting toxicities and overall safety data before dose-escalation or expansion decisions.

---

## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** Safety Analysis Set (all patients who receive at least one dose of study treatment); Efficacy Analysis Set (all dosed patients with measurable disease at baseline). **Sample Size Justification:** ~30 to 60 patients enrolled per specific treatment arm to robustly characterize the safety profile and detect preliminary efficacy signals for the combinations. Total actual enrollment: $N=175$. **Handling of Missing Data:** Standard oncology guidelines applied; Kaplan-Meier methods used for time-to-event endpoints (PFS, DoR) with censoring for patients without an event or lost to follow-up.

---

## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular on-site and remote monitoring visits to ensure protocol adherence, data accuracy, and patient safety. **Audit Trail:** Validated Electronic Data Capture (EDC) system with full audit trails, eCRF locking, and automated data clarification forms.

---

## 11. Study Identifiers & Governance

**Primary ID:** {{D933IC00001}} **Registry Number:** {{NCT03819465}} **Posting ID:** {{987019807463995285}} **Sponsor/Lead Organization:** {{AstraZeneca}} **Brief Study Title:** {{A Study of Novel Anti-cancer Agents in Patients With Previously Untreated NSCLC}} **Official Regulatory Title:** {{A Phase IB, Open-Label, Multi-Center Study to Determine the Efficacy and Safety of Durvalumab and/or Novel Oncology Therapies, With or Without Chemotherapy, for First-Line Stage IV Non-Small Cell Lung Cancer (NSCLC)}}

---

## 12. Clinical Framework (NSCLC Specific)

**Disease Area / Primary Condition:** {{Metastatic Non-Small Cell Lung Cancer (NSCLC)}} **Preferred UMLS Name:** {{Non-small cell lung carcinoma metastatic}} **Diagnosis Threshold:** {{Histologically or cytologically documented Stage IV NSCLC}} **Medical Methodology:** {{Immune Checkpoint Inhibition combined with Novel Agents / Chemotherapy}} **Optimization Target:** {{Objective Tumor Response and Prolonged Progression-Free Survival}}

---

## 13. Study Procedures & Methodology

**Management Pathway:** {{First-line targeted immunotherapy / chemotherapy standard oncological pathway}} **Education Protocol (HCP):** {{Investigator Meetings and Site Initiation Visits (SIV) for safety/DLT monitoring}} **Education Protocol (Patient):** {{Informed Consent process, management of immune-related adverse events (irAEs)}} **Quality Audit Mechanism:** {{SRC evaluation, site monitoring, and central RECIST independent review mechanisms}}

---

## 14. Observation & Follow-Up Schedule

**Total Duration:** {{Up to progression, unacceptable toxicity, or End of Study (~7 years max total study window)}} **Onsite Visit Cadence:** {{Day 1 of every 3-week or 4-week treatment cycle}} **Remote Monitoring Frequency:** {{As needed between treatment cycles and during survival follow-up}} **Checklist Review Interval:** {{Tumor imaging assessments scheduled every 6-8 weeks}}

---

## 15. Sign-off & Approval

| Role | Name | Signature | Date |
| :--- | :--- | :--- | :--- |
| **Principal Investigator** | [Name] | ____ | [DD-MMM-YYYY] |
| Clinical Operations Lead | [Name] | ___ | [DD-MMM-YYYY] |
| Lead Statistician | [Name] | _____ | [DD-MMM-YYYY] |